

Clarity Health — Pitch Brief

A communication layer between GPs, patients, and allied health practitioners. Built in Australia.

The one-liner

Patients leave the GP confused. Allied health practitioners chase context they never get. Clarity Health bridges all three — turning every consult into a plain-English summary the patient understands and the next practitioner can build on.

The problem

- 75% of medical information is forgotten by patients before they leave the room. 50% of what they do remember is recalled incorrectly.
- The average 20-minute Medicare-rebated consult is too short to explain conditions, medications, side-effects, and next steps in language patients absorb.
- When that patient sees a physio, psych, dietitian, or OT next, they re-tell their story from scratch. The allied health practitioner makes decisions without the GP's notes, medications, or referral context.
- The existing tools — My Health Record, Heidi, Halo — solve the doctor's note-taking problem. Nobody is solving the patient-comprehension problem or the cross-practitioner continuity problem.

The solution

The core loop:

1. GP records or pastes a consult transcript. Either via the in-app voice recorder or by pasting from Heidi into our Chrome extension.

2. **AI summarises** the consult — plain English, action items, medications with PBS info, referrals, glossary of medical terms, urgency flags.
3. **GP reviews and clicks Approve.** Non-negotiable — nothing reaches the patient until the GP has signed off.
4. **Patient receives the approved summary** on their device. They can ask follow-up questions through an embedded medical AI scoped to that specific visit (with a hard "this is not medical advice" disclaimer on every reply).
5. **Allied health continuity.** When the patient sees their physio next, the practitioner opens the patient's profile and gets an AI-generated health-status summary — recent visits, conditions, current medications. The physio can record their own consult, which joins the chain. The GP doesn't need to be involved for the loop to keep going.

The product — four surfaces

Surface	User	What they do
GP web app	GPs, clinic staff	Record consult OR paste transcript → review AI summary → approve → patient receives
GP Chrome extension	GPs already using Heidi or other transcription	Paste raw transcript into extension popup → AI summarises → approve → patient receives. Slots into existing workflows.
Patient app	Patients	Receive approved summaries, chat with the embedded medical AI, manage their care team, share access with new practitioners
Allied health portal	Physios, psychs, dietitians, OTs	View patient profile (AI summary + recent visits), record their own consult under the patient

The join primitive — practitioner-initiated QR / link

The piece that makes the network grow:

- Every practitioner generates a personal QR code + invite link.
- Patient scans/clicks → "Add Dr X to my care team" → done. If new, signup carries the invite through onboarding.

- Practitioner now sees the patient on their dashboard. Can record consults. Patient receives approved summaries.
- Same primitive works in every channel — printed card at reception, email signature, SMS follow-up, allied-health business card.

Why this matters for distribution. A patient who lands on the app from a GP's QR becomes a viral node — they take Clarity Health with them when they see their physio, who joins the same network. **Cross-practitioner network effects without needing a top-down government rollout.**

Consent + privacy (built in, not bolted on)

Three layers, each maps to a real-world consent moment:

1. **Care Team (whitelist)** — patient adds practitioner via QR; ongoing access until revoked.
2. **Practitioner-initiated invite link** — the join primitive above. Designed for the way clinics actually work — practitioners hand out cards, patients scan.
3. **Audit log (mandatory)** — every practitioner read of a patient's profile or visit is logged. Patient can see the full log. Required under the Privacy Act 1988.

Patient transcripts are sensitive health information under Australian privacy law. Storage is user-scoped, audio blobs aren't returned to the client after upload, raw transcripts are gated behind a separate per-practitioner toggle.

Why Australia

- **Single-payer Medicare structure** — every GP consult has the same time pressure, same item numbers (23, 36, 44), same incentives. One product fits the whole market.
 - **PBS (Pharmaceutical Benefits Scheme)** — medication explanations can reference PBS subsidy directly, which means more to Australians than generic drug info.
 - **AHPRA** — every registered practitioner has a unique ID. Verification can be automated as we scale. No equivalent in the US.
 - **Existing trust signal** — the "after-visit summary" concept is universal but never been done well for the patient side in Australia. Heidi has proven AU GPs will adopt AI tools fast.
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Tech stack (built for speed)

- **Frontend** — React + TypeScript + Vite, Tailwind, shadcn/ui
 - **Auth + DB** — Supabase Postgres with row-level security
 - **Transcription** — Deepgram nova-2-medical, en-AU, with speaker diarisation
 - **AI summarisation** — Google Gemini 2.5 Flash (with structured JSON output)
 - **Chrome extension** — Manifest v3, sits alongside Heidi without disrupting workflow
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What's built today (May 2026)

- GP web app — recording, paste-transcript, AI summarisation pipeline, approval flow
 - Practitioner accounts with AHPRA registration + clinic linkage
 - **Practitioner-initiated QR / care-team join flow** (just shipped) — the network primitive
 - Patient app — dashboard, visit summaries with all nine sections (quick summary, urgency flags, action items, recommendations, medications, referrals, key points, glossary, follow-up questions)
 - Patient-recorded fallback (clearly marked unverified) for patients whose GP isn't on the platform yet
 - Audit log infrastructure
 - Demo mode on every page for sales conversations
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What's next (roadmap)

Phase	Scope
Phase 1.5	Push / SMS / email notifications when a summary is approved
Phase 2	Chrome extension public release + visit-attached claim link for un-onboarded walk-in patients
Phase 3	Patient-side Care Team management UI + audit log view + revoke
Phase 4	Full allied-health portal — patient list, profile view, record-under-patient
Phase 5	Automated AHPRA verification at signup

Deferred deliberately — Stripe billing, family sharing, multi-language, government integrations, emergency break-glass override. None block the core loop.

Business model

Current posture — under review post-pivot. Originally patient subscription. Now exploring clinic-side billing (per-practitioner SaaS, similar to Heidi's pricing) plus a free / freemium patient tier.

Why clinic-led is right:

- Clinics already pay for software (Best Practice, Medical Director, Heidi).
 - A practice with 8 GPs paying \$50/seat/month = \$400 MRR. **150 clinics = \$60k MRR.**
 - Patient adoption is bundled with the clinic decision, not a separate sales motion.
 - Allied health practitioners can be invited free initially to build the cross-practitioner network, then converted to paid as their patient volume grows.
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Why now

- **GPs are already adopting AI** — Heidi proved it. Halo, Patient Notes, Lyrebird are growing. The wedge for AI tools in Australian primary care is open.

- **Patient-facing AI is finally trustworthy enough.** Gemini 2.5 Flash + medical-grade Deepgram transcription means summaries are actually accurate. Two years ago this wasn't possible.
 - **Allied health is digitised but disconnected.** Practitioners are on Cliniko, Halaxy, Power Diary — none of them talk to each other or to GPs.
 - **Privacy Act 1988 reforms** are tightening — products built privacy-first now will be the ones standing in 18 months.
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Distribution wedge

GP clinics, one at a time. Land 5–10 anchor clinics in Melbourne, prove the loop (patients understand more, allied health gets context, GPs save time on follow-up calls), expand by referral. Allied health is invited free → upgraded once they're the ones recording.

The Chrome extension is the *trojan horse* — it sits next to Heidi, doesn't replace it, and gives GPs a "send to patient" button they don't have today.

Team

- **Founder** — Ivan Htet. Building solo today, looking to bring on a clinical co-founder (GP advisor) and a sales/clinic-onboarding hire after first 5 paying clinics.
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What we need (the ask)

This brief is for you, Claude, to help build a pitch deck. The deck should hit:

1. **Hook slide** — one sentence + visual that captures "the gap between leaving the GP's office and understanding what just happened."
2. **The problem** — make it visceral. The 75% / 50% statistics, the allied-health continuity gap.
3. **The product** — the four surfaces, but presented as one connected loop, not four products.
4. **Demo** — screenshots of the patient-facing summary (the nine sections). This is the wow.
5. **The join primitive** — the QR/invite flow as the network-growth mechanism.
6. **Why Australia, why now** — Medicare structure + Heidi proof point + Privacy Act tailwind.

7. **Business model** — clinic-side SaaS, with the math.
8. **Roadmap** — what's built, what's next, what's deferred.
9. **Team** — founder + ask.
10. **Close** — vision: every Australian GP visit produces a summary the patient actually reads.